



February 23, 2026

Dr. Thomas Keane
Assistant Secretary for Technology Policy
U.S. Department of Health and Human Services
Attention: RIN 0955-AA13
7033A, 330 C Street SW
Washington, DC 20201
Via electronic submission to: <https://www.regulations.gov>

Re: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Dear Assistant Secretary Keane:

Baylor Scott & White Health (BSWH) views artificial intelligence (AI) as a critical enabler of safer, higher-quality, and more efficient clinical care. In our experience as the largest not-for-profit health system in Texas, BSWH has successfully embedded AI to improve quality and to surface actionable insights in a timely manner. These capabilities are increasingly required for delivering highly reliable care, succeeding under value-based payment models, and providing a customer-centered experience.

While AI technologies have matured rapidly, technology readiness is no longer the primary limiter to adoption. Instead, BSWH's real-world experience shows that progress is most often constrained by four key barriers: regulatory uncertainty, misaligned reimbursement, fragmented data, and establishing trust. These barriers delay the transition of AI solutions from promising pilots into sustained clinical operations. We believe HHS's focus on regulation and reimbursement, as well as specific issues like legal accountability, interoperability, governance, and patient trust is exactly the right approach.

Our recommendations below are intended to help establish a policy environment that protects patients while accelerating innovation.

Create Risk-Based AI Regulation

Federal policy should clearly differentiate clinical vs. non-clinical AI and provide oversight proportional to patient risk. For example, an algorithm providing diagnostic or treatment recommendations warrants more stringent oversight than a lower-risk administrative AI that improves documentation. This stratification would ensure patient safety without over-burdening lower-risk uses of AI.

Further, ambiguity around liability and accountability for non-FDA-regulated AI tools (e.g. clinical decision support algorithms not classified as medical devices) is creating hesitation among legal, compliance, and clinical leaders in health systems. As highlighted in RFI Question 3, novel legal issues such as liability, indemnification, and privacy obligations for AI require attention. We encourage HHS to work with federal partners on clear guidance that defines accountability for AI use in clinical care, so that providers can innovate with confidence.

Finally, harmonized federal standards and a unified regulatory approach will help prevent a patchwork of state-by-state rules that slow innovation and drive up costs. HHS can play a convening role to align regulations and guidance, reducing duplicative or conflicting requirements while maintaining robust patient safety and privacy protections.

Establish Reimbursement Pathways to Catalyze AI Adoption

Misaligned payment models are a significant barrier to scaling AI in clinical care. As HHS notes in the RFI's Reimbursement section, legacy fee-for-service payment often fails to reward high-value innovations. Even if an AI solution demonstrably improves patient outcomes, it is difficult to move beyond

small pilot projects if there is no mechanism to get reimbursed for its use. BSWH recommends public payers like Medicare and Medicaid should lead in creating reimbursement pathways for AI solutions.

We recommend that CMS develop reimbursement mechanisms (e.g. new or revised CPT codes, add-on payments, or inclusion of AI-based services in value-based care incentive programs) that explicitly cover clinically validated AI interventions. These financial signals are critical to mainstream AI as a standard component of care delivery, similar to how temporary policy changes spurred the rapid expansion of telehealth during the COVID-19 pandemic. Modernizing payment policies in this way will give providers and developers a business case to invest in AI for high-value use cases.

For example, if an AI-powered diagnostic tool can prevent costly complications, Medicare could provide bonus payments or shared savings in value-based arrangements to reward its use. In short, HHS should leverage reimbursement levers to reward proven AI tools that improve outcomes and experiences for patients and providers.

Strengthen Data Sharing and Interoperability Foundations

Interoperability is foundational to effective and scalable clinical AI. Advanced AI algorithms require access to complete, longitudinal patient data across care settings to perform optimally. Unfortunately, health systems continue to face fragmented data environments that hamper AI performance. In response to RFI Question 8 on interoperability, we emphasize that HHS should prioritize policies and standards that break down data silos and promote seamless, secure data exchange in healthcare.

Key steps include accelerating the adoption of FHIR-based APIs for EHR integration, standardizing risk-factor data across government programs, and creating consistent performance measurement standards. These actions would greatly expand the useful data available to AI developers and clinicians. For example, an AI-driven risk predictor is far less effective if a patient's specialist visit records and/or community health data are not accessible to the algorithm.

Promote Transparency, Evaluation, and Trust in AI-Enabled Care

Building trust in AI is essential for both clinicians and patients. In our view, trust will come from rigorous measurement, transparency about AI use, and appropriate oversight mechanisms. We echo the intent of RFI Question 4, effective evaluation methods and post-deployment monitoring are critical to ensure AI tools are safe and adoption accelerates.

BSWH recommends HHS champion the development of standardized metrics for pre- and post-deployment evaluation of AI in clinical settings. Defining clear performance metrics upfront and tracking real-world impact helps identify issues early and fosters confidence among clinicians using AI. HHS could facilitate this by collaborating with industry to create benchmark datasets, testing frameworks, or even voluntary reporting programs for AI performance in the field.

Transparent communication and appropriate governance are cornerstones of trust. Clinicians and patients should be informed when AI is being used in their care. Also, clinician oversight must be preserved. AI should support, not replace, the judgment of healthcare professionals. These principles align with patient and provider expectations, as noted in RFI Question 9, that AI's role be transparent and that humans remain accountable for care.

BSWH also recommends that HHS support private-sector efforts to bolster trust, per Question 5 on accreditation and certification. For example, voluntary accreditation or certification frameworks for clinical AI could help validate products against agreed-upon safety and efficacy criteria, giving health systems confidence in selecting AI tools. Any such programs should be designed in partnership with industry and healthcare stakeholders to ensure they protect patients without unduly slowing innovation.

Conclusion

BSWH firmly believes that AI can be a transformational asset in clinical care when the right policy environment is in place. The major barriers identified in RFI Question 1 – regulatory uncertainty, lack of reimbursement, and data silos closely mirror the challenges BSWH has observed in practice.



We commend HHS for seeking actionable, experience-based input on these issues. By clarifying risk-based AI regulations, aligning reimbursement to reward high-value AI, investing in data interoperability, and establishing trust through transparency and standards, HHS can powerfully accelerate the safe and effective use of AI in clinical care.

BSWH's experience shows that these steps are critical to move AI tools from the pilot stage to deployed solutions that improve patient outcomes and experience in the real world. We stand ready to support HHS in this endeavor and offer our health system's ongoing experience as a testbed for responsible AI innovation. Together, we can ensure that transformative AI technologies are rapidly scaled in ways that advance convenience, outcomes, and experience for our clinicians and customers.

Sincerely,

A handwritten signature in black ink, appearing to read "Lisa Athens", written over a horizontal line.

Lisa Athens, J.D.
SVP Chief Risk Officer



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